

INSTITUTIONAL ENGAGEMENT • A STOP-LOSS INTELLIGENCE ENGAGEMENT

Stop-Loss Placement Study

A Stop-Loss Intelligence Engagement

NET-COST OPTIMAL ATTACHMENT • \$50,000

PREPARED BY KINCAID HEALTH STRATEGIC ADVISORY — A STOP-LOSS INTELLIGENCE ENGAGEMENT PANEL

This study is generated by the Kincaid Health StopLoss IQ engine from the demonstration plan profile. All figures are MODELED until a claims census and carrier quotes are ingested. It is an institutional decision-support deliverable, not actuarial certification. For licensed broker use only.

01 · PLAN PROFILE & EXECUTIVE READ

MODELED

450

EMPLOYEES

\$4,100,000

ANNUAL CLAIMS

\$759

PMPM

\$-2,505,920

RESERVE GAP

The plan currently carries a specific attachment of **\$29,000** against an annual claims base of **\$4,100,000**. Across the four modeled placement tiers, the net-cost-optimal structure is the **aggressive** tier at a **\$50,000** specific attachment: expected recoveries of \$580,000 against premium of \$312,000 produce the lowest modeled net cost of **\$3,832,000**. The IBNR run-out position shows a reserve gap of **\$-2,505,920** that should be funded before renewal.

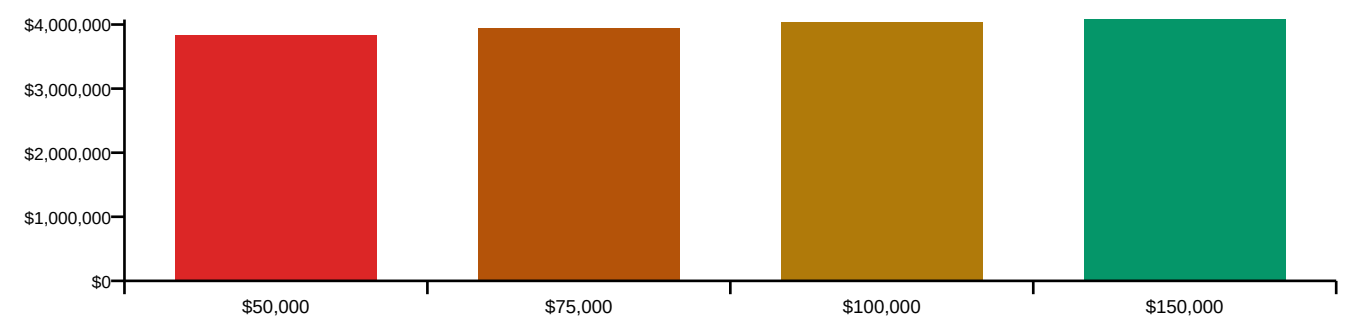
02 · ATTACHMENT POINT ANALYSIS — FOUR PLACEMENT TIERS

MODELED

Tier	Specific Attachment	Premium	Expected Recoveries	Max Exposure	Net Cost
Aggressive	\$50,000	\$312,000	\$580,000	\$22,500,000	\$3,832,000
Balanced	\$75,000	\$228,000	\$390,000	\$33,750,000	\$3,938,000
Conservative	\$100,000	\$178,000	\$250,000	\$45,000,000	\$4,028,000
High-Deductible	\$150,000	\$118,000	\$140,000	\$67,500,000	\$4,078,000

MODELED NET COST BY ATTACHMENT TIER

MODELED



03 · CATASTROPHIC CLAIMANT WATCHLIST

MODELED

Member	Diagnosis	YTD Cost	Projected Run-Out	Breaches Attachment	Recovery Est.	Severity
M-0047	Leukemia (AML)	\$1,240,000	\$1,840,000	YES	\$990,000	CRITICAL
M-0183	Premature Birth (32 weeks)	\$480,000	\$620,000	YES	\$370,000	HIGH
M-0291	Spinal Fusion (3-level)	\$210,000	\$240,000	no	\$0	MEDIUM
M-0312	Hepatitis C (Harvoni)	\$89,000	\$184,000	no	\$0	WATCH

Member	Diagnosis	YTD Cost	Projected Run-Out	Breaches Attachment	Recovery Est.	Severity
M-0401	ESRD — Dialysis	\$68,000	\$272,000	YES	\$172,000	HIGH

Modeled reimbursement pipeline across breaching claimants: **\$1,532,000**. Each breaching claimant should be filed with the carrier before the run-out window closes.

04 · IBNR & RUN-OUT RESERVE POSITION

MODELED

Component	Amount
Total incurred claims	\$4,100,000
IBNR estimate (8.3% factor)	\$340,300
Run-out window	3.2 months
Recommended reserve	\$1,184,080
Current reserve	\$3,690,000
RESERVE GAP	\$-2,505,920

05 · SPECIALTY RISK CONCENTRATION

MODELED

Specialty	Claims	Members	Risk Level
Oncology	\$1,420,000	3	CRITICAL
Neonatology	\$480,000	1	HIGH
Orthopedics	\$380,000	12	MEDIUM
Nephrology	\$310,000	4	HIGH
Cardiology	\$270,000	8	MEDIUM
Mental Health	\$190,000	34	WATCH

06 · CARRIER MARKET COMPARISON — FIVE QUOTES

MODELED

Carrier	Specific	Agg Factor	Premium	Rated Claims	AM Best	Share
Sun Life Financial	\$75,000	115%	\$228,400	\$4,180,000	A+	18.2%
HM Life Insurance	\$75,000	115%	\$241,200	\$4,050,000	A	11.4%
Symetra Life	\$75,000	120%	\$218,800	\$4,320,000	A	9.7%
Voya Financial	\$80,000	115%	\$201,600	\$4,240,000	A-	7.8%

Carrier	Specific	Agg Factor	Premium	Rated Claims	AM Best	Share
Tokio Marine HCC	\$75,000	110%	\$265,000	\$3,980,000	A++	14.1%

Lowest quoted premium: **Voya Financial** at \$201,600 (AM Best A-). Premium alone is not the decision rule: rated claims, aggregate factor and carrier claims-payment posture must be weighed together.

07 · EVIDENCE & CONFIDENCE TIER

DOCTRINE

Every figure in this study is **MODELED**: the plan profile, claimant watchlist and carrier quotes are demonstration fixtures pending ingestion of the plan's claims census, carrier quote sheets and reserve statements. Ingesting those artifacts would move the attachment analysis and IBNR position to CERTIFIED. This document carries a SHA-256 evidence manifest in its download headers (Hard Rule 03).